MEDRECORDS

Business Model & Go-To-Market Strategy

How MedRecords makes money, who pays first, and the sequence of expansion

1. Commercial Thesis

MedRecords is a multi-sided platform. Patients carry their records, providers do clinical work on the records, and payers and government consume the resulting structured data. We do not monetize patients. We monetize the providers and payers whose workflows we improve and whose data exhaust is valuable.

The strategic question is sequencing: in a market with low willingness-to-pay across most segments, which segment do we monetize first to fund the rest, and how do we structure incentives so that each new segment increases the value of the platform to the others?

Our answer is: free for patients, freemium for pharmacies, paid SaaS for clinics, and per-member-per-month for payers. Pharmacies are the wedge. Payers are the long-term revenue base.

2. Market Sizing

2.1 Top-Down View (Nigeria)

Indicator	Magnitude
Population	~225M people
NIN-registered	~115M (sufficient base for an NIN-anchored product)
Annual healthcare spend (public + private)	Estimated USD 12–15B
Registered pharmacies (PCN)	~6,000+ retail premises
Registered private hospitals & clinics	~30,000+ (highly fragmented; many sub-scale)
HMOs (NHIA-accredited)	~80
Insurance penetration	~5% but climbing with state schemes and NHIA expansion

Numbers above are working estimates from public sources for sizing only. Pre-investment work should triangulate against PCN, MDCN, NHIA, and NBS data.

2.2 Serviceable Market (5-Year View)

- Tier-1 cities (Lagos, Abuja, Port Harcourt, Ibadan, Kano): the addressable adoption base. Higher provider density, better connectivity, more insured lives.
- Pharmacies: an addressable base in the low thousands once we are operating across Tier-1 cities.
- Private clinics and hospitals: addressable base in the high tens of thousands long-term; concentrated initial target of a few hundred.
- HMOs and state insurance schemes: an addressable pool of dozens of institutional payers, each representing tens or hundreds of thousands of covered lives.

3. Customer Segments & Value Propositions

3.1 Patients

Free for individuals. Premium family tier as an optional upsell.

Value: Lifetime portable record, faster care, fewer repeated tests, embassy-ready exports, prescription safety, dependent management.

Why free: Patient adoption is the platform's defensibility. Friction at the patient layer destroys network effects. Charging individuals would also expose us to regulatory and reputational risk around access to health records.

3.2 Pharmacies

Value: Prescription verification, duplicate-dispense detection, controlled substance compliance, drug interaction alerts, customer loyalty through refill flow, dispute reduction.

Why pharmacies first: Highest transaction frequency per location, simplest workflow integration, clearest compliance pull (NAFDAC controlled-substance reporting), and the data they generate (dispenses) is what makes the platform valuable to GPs in the next wave.

3.3 GP Clinics & Small-to-Mid Hospitals

Value: Faster patient onboarding, accurate history, fewer repeat tests, structured referrals, retention of patients within their network, fewer prescription disputes with pharmacies.

Sequencing: Approached after pharmacy density in a target area reaches a threshold, at which point we can demonstrate "when you prescribe, you can see what was dispensed; when a new patient walks in, you can see what they were prescribed elsewhere."

3.4 Specialists, Dental, Optical, Labs

Value: Inbound referrals from GPs already on the platform; specialty-scoped record visibility; structured reporting back to the referring physician.

Sequencing: Onboarded opportunistically alongside GP rollout; full push starts once GP density justifies the network effect for them.

3.5 Large Hospitals & Hospital Groups

Value: Compliance-ready records, NHIA claims acceleration, internal patient continuity across departments, reduced repeat investigations.

Sequencing: Late v1 / early v2. These are long sales cycles requiring deep HMS integration. We approach them once payer pressure ("this scheme requires MedRecords-compatible records") creates top-down pull, and once we have HMS adapters for their existing systems.

3.6 Payers (HMOs, NHIA, State Schemes)

Value: Structured claims-grade clinical data, fraud reduction, faster adjudication, risk-pricing data, population health insight.

Sequencing: First payer pilot in v1 with one HMO. Becomes the largest revenue line by year 3 because their willingness-to-pay scales with covered lives and our data improves their loss ratio.

3.7 Government & Public Health

Value: Disease surveillance, immunization coverage, drug supply chain insight, donor-funded program reporting.

Sequencing: Indirect for v1 — we engage early on standards and compliance but do not depend on government revenue. Direct partnerships (e.g., Federal Ministry of Health, state ministries) become real in v2+, often funded by international donors (Gates Foundation, USAID PEPFAR, the Global Fund).

4. Pricing Strategy

All pricing below is illustrative for planning. Final numbers require beta validation. NGN figures use a long-run reference rate; USD is shown for international reference. We expect substantial elasticity testing in months 6–12.

4.1 Patient Pricing

Tier	Inclusions
Free	All core features: record, consent, audit, single export per quarter, dependent management for up to 3 dependents
Premium (Family) — ~NGN 2,000/month or NGN 20,000/year	Unlimited exports, up to 10 dependents, 24/7 priority support, expedited NIN re-verification, multi-language support enhancements

4.2 Pharmacy Pricing

Tier	Price (NGN/month)	Inclusions
Free Tier	0	Up to 200 dispenses/month, basic verification, no analytics, MedRecords branding visible
Standard	~NGN 15,000/month per branch	Unlimited dispenses, drug interaction alerts, refill reminders, basic analytics
Premium	~NGN 35,000/month per branch	Above + multi-branch dashboard, controlled-substance compliance pack, refill SMS to customers, branded patient touchpoint

4.3 Clinic & Hospital Pricing

Segment	Price (NGN)	Inclusions
Solo Practitioner	~NGN 8,000 / clinician / month	Single clinician; full encounter authoring; templates; basic exports
Clinic	~NGN 6,000 / clinician / month (min 3)	Multi-clinician; referrals; specialty modules; admin console
Hospital	Custom (volume + integration)	All above + HMS integration; SLA; dedicated CSM; on-prem cache option

4.4 Payer Pricing

Customer	Pricing approach	Inclusions
HMO / State Scheme	Per-member-per-month (PMPM), tiered by volume	Read access to enrolled members' encounters with member consent; analytics dashboards; claims-grade exports
NHIA	Negotiated framework agreement	Volume PMPM with national pricing; reporting obligations to NHIA in return

Indicative PMPM range: $0.20 to $0.80 per enrolled member per month, depending on scheme size, integration depth, and analytics tier. This is a long-term recurring revenue line and the primary driver of unit economics by year 3.

4.5 API / Integration Partners

Partner type	Approach	Notes
HMS Vendors	Revenue share (10–20%) on MedRecords seats sold via their channel	Co-marketing, integration support
Lab Networks	Free read of orders; free write of results; paid API for high-volume custom integrations	Strategic — labs are nodes we need on the platform
Third-party developers	Usage-based metered API access above free quota	Public API with rate limits

5. Unit Economics (Illustrative)

These are planning estimates, not financial guidance. They are sized to test whether the model can work, not to forecast outcomes.

5.1 Pharmacy Unit

Metric	Estimate
ARPU	~NGN 20,000/month blended across tiers
CAC	~NGN 60,000 (3 months ARPU) — field sales + onboarding
Gross margin	~75% (hosting, integration support, transaction infra)
Payback period	Approximately 4 months
Annual logo churn target	Under 15%

5.2 Clinic Unit

Metric	Estimate
ARPU	~NGN 25,000/clinician/month
CAC	~NGN 100,000 (4 months ARPU) — longer sales cycle
Gross margin	~70%
Payback period	Approximately 6 months
Annual logo churn target	Under 10% (sticky once integrated into workflow)

5.3 Payer Unit

Metric	Estimate
ARPU per scheme	Highly variable — six to seven figures USD annually for large schemes
CAC	Long enterprise sale: 9–15 months; high touch
Gross margin	~85% (mostly analytics + read access on existing data)
Net revenue retention target	Over 120% (more lives, more modules)

6. Go-To-Market Plan

6.1 Geographic Sequencing

- Phase 1 (months 0–9): Two LGAs in Lagos (e.g., Lekki/Eti-Osa and Ikeja). High provider density, high smartphone penetration, manageable footprint for field sales.
- Phase 2 (months 9–15): Expand across Lagos State and add Abuja.
- Phase 3 (months 15–24): Add Port Harcourt, Ibadan, Kano. Begin payer-led national distribution via NHIA-accredited schemes.
- Phase 4 (year 3+): Cross-border expansion within ECOWAS, starting with Ghana, which has comparable health infrastructure and a working national ID program.

6.2 Acquisition Motions by Segment

Segment	Motion
Pharmacies	Field sales (boots-on-ground reps walking the area); PCN association partnership; referral program from existing pharmacy customers
GP Clinics	Field sales after pharmacy density established; MDCN association partnership; webinars on NDPA compliance; case studies from pharmacy data
Specialists	Inbound from GP referrals; specialty association partnerships (Nigerian Dental Association, etc.)

Segment	Motion
Hospitals	Enterprise sales with named accounts; founder-led for the first 5; CSM-led thereafter
HMOs	Founder-led enterprise sale; warm intros through investor and advisor networks; framework deals with NHIA
Patients	Driven entirely by provider distribution. When a patient visits a MedRecords-enabled pharmacy or clinic, they are invited to claim their record. Limited direct-to-consumer marketing in v1.

6.3 Partnerships That Matter

- NIMC — for production NIN verification API access. Critical-path partnership.
- Mobile network operators (MTN, Airtel, Glo, 9mobile) — for zero-rated patient app data and USSD shortcodes.
- PCN, MDCN, ODORBN — professional registries for provider verification and association endorsements.
- Top 2–3 Nigerian HMS vendors — for adapter integrations.
- 1–2 leading HMOs — for design-partner relationship to validate payer use case.
- Donor agencies (Gates, USAID, AfDB) — for grant funding tied to public health outcomes.

7. Funding Plan

Stage	Target	Use
Pre-seed / Seed	USD 2–3M	Months 0–12. Build MVP, Lagos Phase 1 launch, 50 active providers, first payer pilot.
Series A	USD 8–12M	Months 12–30. Multi-city expansion, payer revenue ramp, HMS integrations, regulatory certifications.
Grant funding (non-dilutive)	USD 1–3M	Tied to public health outcomes — vaccination tracking, antenatal care, disease surveillance. Pursued in parallel.
Series B	USD 25M+	Year 3+. National coverage, ECOWAS expansion, payer-driven distribution at scale.

8. Competitive Landscape

8.1 What We Are Not Competing With

- HMS vendors (e.g., institutional ERP-style systems) — we integrate with them, we do not replace them.
- Telemedicine startups (Reliance Health, Helium Health Reliance, Mobihealth, etc.) — they own the consultation flow; we own the record layer.
- Pharmacy POS vendors — we integrate above their layer.

8.2 Where Competitive Pressure Will Come From

- Helium Health and similar HMS-adjacent companies if they pivot toward cross-institutional records.
- Mobihealth and Reliance Health if they extend their PHR features beyond their care networks.
- An eventual government-mandated national EHR (which is plausible within 5 years; this is why standards alignment and government engagement matter).
- International players (Epic, Cerner, regional Indian players) entering Africa — unlikely in our window but watchable.

8.3 Defensibility

- Network effects: the more providers and dispenses on the platform, the more valuable for any new provider to join.
- Patient ownership: once a patient has a verified record with MedRecords, switching costs are high.
- Standards leadership: by publishing the Nigeria FHIR Implementation Guide, we shape the ecosystem.
- Regulatory relationships: deep NDPC and FMOH engagement is hard to replicate quickly.
- Data: aggregate de-identified data becomes valuable to payers, public health, and research — a moat that grows with time.

9. Key Assumptions to Validate

- Pharmacies will pay NGN 15K+/month for verification and analytics — to validate in months 3–9 of beta.
- Patients will adopt at meaningful rates when invited at the point of care — measured by claim rate from provider invites.
- NIN verification API performance is reliable enough for real-time onboarding — to confirm with NIMC during partnership negotiation.
- HMOs will pay PMPM for clinical data — first design partner sale validates this; failure here is the largest commercial risk.
- Provider time-to-first-record is under 15 minutes — measured during pilot, dictates onboarding economics.